HEALTH SYSTEMS, PUBLIC HEALTH AND UNIVERSAL HEALTH COVERAGE

COVERAGE CUBE → POLICY AND MISSION CARD → TWO PILLARS KEPT APART → TWO ENROLMENT ROUTES → SOURCE DISCIPLINE CARD → IMPOVERISHMENT MECHANISM

Read the thick cyan rail from Stage 00 to the final core synthesis. Each card uses a concept-appropriate structure; the grey E card is optional enrichment only and never repairs missing core content.

◆ PRIMARY CORE FLOW · ◈ SUBORDINATE EXTRA · ◆ APPROVAL / REVIEW

CORE BEFORE EXTRA • prior artefacts unchanged • exact same master drives poster and tiles

Approval: FALSE • Pending user review • source generation g3 and all prior artifacts unchanged

00

STAGE 00 COVERAGE CUBE

COVERAGE CUBE · WHO IS COVERED, ALL CITIZENS OR IDENTIFIED BENEFICIARIES · PROMOTIVE, PREVENTIVE, CURATIVE AND REHABILITATIVE CARE · FINANCIAL PROTECTION · HOW LITTLE THE HOUSEHOLD PAYS AT THE POINT OF · A SYSTEM CAN WIDEN ONE EDGE WHILE NARROWING ANOTHER · MUST REMEMBER · UNIVERSAL HEALTH COVERAGE REQUIRES PROMOTIVE, PREVENTIVE, CURATIVE

CORE CLAIM

- POPULATION → breadth: who is covered, all citizens or identified beneficiaries
- SERVICES → depth: promotive, preventive, curative and rehabilitative care

MECHANISM

- FINANCIAL PROTECTION → height: how little the household pays at the point of care
- RULE → a system can widen one edge while narrowing another; test all three

CONSEQUENCE / LIMIT

- MUST REMEMBER: Universal health coverage requires promotive, preventive, curative,...
- POPULATION → breadth: who is covered, all citizens or identified beneficiaries

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: RULE → a system can widen one edge while narrowing another; test all three.

01

STAGE 01 POLICY AND MISSION CARD

POLICY AND MISSION CARD · POLICY 2017 · RAISE PUBLIC HEALTH EXPENDITURE TO 2.5 PER CENT OF · ITS STATUS · A POLICY DOCUMENT, NOT AN ENACTED STATUTE; COMMITMENTS, NOT · RURAL MISSION · LAUNCHED 12 APRIL 2005 FOR THE RURAL POPULATION AND · URBAN MISSION

POLICY 2017 → raise public health expenditure to 2.5 per cent of gross domestic product → ITS STATUS → a policy document, not an enacted statute; commitments, not entitlements → RURAL MISSION → launched 12 April 2005 for the rural population and vulnerable groups → URBAN MISSION → Union Cabinet decision dated 1 May 2013 created it as a sub-mission → SPECIAL FOCUS → Empowered Action Group States, North East, Jammu and Kashmir, Himachal

CORE CLAIM

- POLICY 2017 → raise public health expenditure to 2.5 per cent of gross domestic product
- ITS STATUS → a policy document, not an enacted statute; commitments, not entitlements

MECHANISM

- RURAL MISSION → launched 12 April 2005 for the rural population and vulnerable groups
- URBAN MISSION → Union Cabinet decision dated 1 May 2013 created it as a sub-mission

CONSEQUENCE / LIMIT

- SPECIAL FOCUS → Empowered Action Group States, North East, Jammu and Kashmir, Himachal
- BENCHMARK → facilities measured against the Indian Public Health Standards

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: BENCHMARK → facilities measured against the Indian Public Health Standards.

02

STAGE 02 TWO PILLARS KEPT APART

TWO PILLARS KEPT APART · PILLAR ONE · WELLNESS CENTRES AND HEALTH MANDIRS; COMPREHENSIVE PRIMARY HEALTH CARE · ITS PACKAGE · TWELVE SERVICES INCLUDING NON-COMMUNICABLE DISEASE AND MENTAL HEALTH · ITS STAFF · COMMUNITY HEALTH OFFICER OR MID-LEVEL HEALTH PROVIDER · PILLAR TWO

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
PILLAR ONE → wellness centres and health mandirs; comprehensive primary health care	ITS STAFF → community health officer or mid-level health provider	ITS QUANTUM → up to five lakh rupees per family per year at empanelled hospitals
ITS PACKAGE → twelve services including non-communicable disease and mental health	PILLAR TWO → cashless secondary and tertiary hospitalisation cover	PILLAR ONE → wellness centres and health mandirs; comprehensive primary health care

CORE CLAIM

- PILLAR ONE → wellness centres and health mandirs; comprehensive primary health care
- ITS PACKAGE → twelve services including non-communicable disease and mental health

MECHANISM

- ITS STAFF → community health officer or mid-level health provider
- PILLAR TWO → cashless secondary and tertiary hospitalisation cover

CONSEQUENCE / LIMIT

- ITS QUANTUM → up to five lakh rupees per family per year at empanelled hospitals
- PILLAR ONE → wellness centres and health mandirs; comprehensive primary health care

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: ITS QUANTUM → up to five lakh rupees per family per year at empanelled hospitals.

03

STAGE 03 TWO ENROLMENT ROUTES

TWO ENROLMENT ROUTES · CORE ROUTE · ELIGIBILITY-BASED, IDENTIFIED THROUGH CENSUS DEPRIVATION CRITERIA · AGE ROUTE · EVERY CITIZEN AGED SEVENTY AND ABOVE MAY ENROL REGARDLESS · THE AGE ROUTE IS UNIVERSAL ONLY FOR THAT AGE · ERROR TO AVOID · WRITING AN AGE-SPECIFIC EXPANSION UP AS UNIVERSAL COVER FOR

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
CORE ROUTE → eligibility-based, identified through census deprivation criteria	PRECISION → the age route is universal only for that age group	CORE ROUTE → eligibility-based, identified through census deprivation criteria
AGE ROUTE → every citizen aged seventy and above may enrol regardless of income	ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages	AGE ROUTE → every citizen aged seventy and above may enrol regardless of income

CORE CLAIM

- CORE ROUTE → eligibility-based, identified through census deprivation criteria
- AGE ROUTE → every citizen aged seventy and above may enrol regardless of income

MECHANISM

- PRECISION → the age route is universal only for that age group
- ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages

CONSEQUENCE / LIMIT

- CORE ROUTE → eligibility-based, identified through census deprivation criteria
- AGE ROUTE → every citizen aged seventy and above may enrol regardless of income

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages.

04

STAGE 04 SOURCE DISCIPLINE CARD

SOURCE DISCIPLINE CARD · HEALTH AUTHORITY · IMPLEMENTS THE HOSPITALISATION PILLAR; EMPANELS AND PAYS CLAIMS · HEALTH ACCOUNTS · OFFICIAL ESTIMATES OF PUBLIC, PRIVATE AND OUT-OF-POCKET SPENDING · 47.1 PER CENT IN 2019-20; 44.4 PER CENT IN · NEVER CONFUSE THE PUBLICATION DATE WITH THE ACCOUNTING YEAR · A HIGH SHARE SIGNALS WEAK PUBLIC PROVISION, NOT HIGH

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims	SEQUENCE → 47.1 per cent in 2019-20; 44.4 per cent in 2020-21; 39.4 per cent in 2021-22	MEANING → a high share signals weak public provision, not high utilisation
HEALTH ACCOUNTS → official estimates of public, private and out-of-pocket spending	RULE → never confuse the publication date with the accounting year	HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims

CORE CLAIM

- HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims
- HEALTH ACCOUNTS → official estimates of public, private and out-of-pocket spending

MECHANISM

- SEQUENCE → 47.1 per cent in 2019-20; 44.4 per cent in 2020-21; 39.4 per cent in 2021-22
- RULE → never confuse the publication date with the accounting year

CONSEQUENCE / LIMIT

- MEANING → a high share signals weak public provision, not high utilisation
- HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: RULE → never confuse the publication date with the accounting year.

05

STAGE 05 IMPOVERISHMENT MECHANISM

IMPOVERISHMENT MECHANISM · CARE IS SOUGHT FROM A PROVIDER OUTSIDE THE FUNDED · OUT-OF-POCKET PAYMENT AT THE POINT OF SERVICE · CATASTROPHIC SPENDING · ABOVE A THRESHOLD SHARE OF HOUSEHOLD INCOME OR CONSUMPTION · ASSET SALE OR DEBT · YEARS OF ACCUMULATION ARE UNDONE BY ONE HOSPITALISATION · THIS IS WHERE THE HEALTH OWNER MEETS THE POVERTY

ILLNESS → care is sought from a provider outside the funded public layer → OUT-OF-POCKET PAYMENT at the point of service → CATASTROPHIC SPENDING: above a threshold share of household income or consumption → ASSET SALE OR DEBT: years of accumulation are undone by one hospitalisation → LINK → this is where the health owner meets the poverty owner

CORE CLAIM

- ILLNESS → care is sought from a provider outside the funded public layer
- OUT-OF-POCKET PAYMENT at the point of service

MECHANISM

- CATASTROPHIC SPENDING: above a threshold share of household income or consumption
- ASSET SALE OR DEBT: years of accumulation are undone by one hospitalisation

CONSEQUENCE / LIMIT

- LINK → this is where the health owner meets the poverty owner
- CLOSE DISTINCTION: UHC is not free hospitalisation alone, insurance coverage is not...

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: CLOSE DISTINCTION: UHC is not free hospitalisation alone, insurance coverage is not.

06

STAGE 06 REFERRAL SPINE

REFERRAL SPINE · FIRST CONTACT; OUTREACH AND COMMUNITY-LEVEL SERVICES · PRIMARY HEALTH CENTRE · PREVENTIVE, PROMOTIVE AND BASIC CURATIVE CARE · COMMUNITY HEALTH CENTRE · SPECIALIST OUTPATIENT AND INPATIENT CARE · DISTRICT HOSPITAL · SECONDARY CARE AND THE GATEWAY TO TERTIARY REFERRAL

SUB-CENTRE → first contact; outreach and community-level services

PRIMARY HEALTH CENTRE → preventive, promotive and basic curative care → COMMUNITY HEALTH CENTRE → specialist outpatient and inpatient care → DISTRICT HOSPITAL → secondary care and the gateway to tertiary referral

CORE CLAIM

- SUB-CENTRE → first contact; outreach and community-level services
- PRIMARY HEALTH CENTRE → preventive, promotive and basic curative care

MECHANISM

- COMMUNITY HEALTH CENTRE → specialist outpatient and inpatient care
- DISTRICT HOSPITAL → secondary care and the gateway to tertiary referral

CONSEQUENCE / LIMIT

- USE → name the failing tier; a vacancy, a diagnostic gap and a specialist shortage differ
- SUB-CENTRE → first contact; outreach and community-level services

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: USE → name the failing tier; a vacancy, a diagnostic gap and a specialist shortage differ.

07

STAGE 07 MARKETISATION AND ITS HARMS

MARKETISATION AND ITS HARMS · PRIVATE DOMINANCE IN CURATIVE CARE · OFTEN UNREGULATED OR UNDER-REGULATED · HIGH COSTS AND UNNECESSARY PROCEDURES · CATASTROPHIC SPENDING AND INEQUITABLE ACCESS · DO NOT DENY PRIVATE CAPACITY, AND DO NOT ASSUME

PRIVATE DOMINANCE IN CURATIVE CARE → often unregulated or under-regulated → HIGH COSTS and unnecessary procedures → CATASTROPHIC SPENDING and inequitable access → BALANCE → do not deny private capacity, and do not assume regulation is automatic

CORE CLAIM

- PRIVATE DOMINANCE IN CURATIVE CARE → often unregulated or under-regulated
- HIGH COSTS and unnecessary procedures

MECHANISM

- CATASTROPHIC SPENDING and inequitable access
- BALANCE → do not deny private capacity, and do not assume regulation is automatic

CONSEQUENCE / LIMIT

- PRIVATE DOMINANCE IN CURATIVE CARE → often unregulated or under-regulated
- HIGH COSTS and unnecessary procedures

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: BALANCE → do not deny private capacity, and do not assume regulation is automatic.

08

STAGE 08 REGULATORY TOOLKIT

REGULATORY TOOLKIT · CLINICAL ESTABLISHMENTS ACT, 2010 · REGISTRATION AND REGULATION OF ESTABLISHMENTS · ITS CONDITION · STATES MUST ADOPT IT OR NOTIFY EQUIVALENT LEGISLATION; ADOPTION · PRICE CAPPING · EXERCISED OVER DEVICES BY THE PHARMACEUTICAL PRICING AUTHORITY · TREATMENT GUIDELINES

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments	PRICE CAPPING → exercised over devices by the pharmaceutical pricing authority	RULE → name the adoption gap a regulatory recommendation is meant to close
ITS CONDITION → States must adopt it or notify equivalent legislation; adoption varies	TREATMENT GUIDELINES → promote rational care and cost control; uptake varies	CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments

CORE CLAIM

- CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments
- ITS CONDITION → States must adopt it or notify equivalent legislation; adoption varies

MECHANISM

- PRICE CAPPING → exercised over devices by the pharmaceutical pricing authority
- TREATMENT GUIDELINES → promote rational care and cost control; uptake varies

CONSEQUENCE / LIMIT

- RULE → name the adoption gap a regulatory recommendation is meant to close
- CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: RULE → name the adoption gap a regulatory recommendation is meant to close.

09

STAGE 09 FINANCING FAMILIES AND CHAIN

FINANCING FAMILIES AND CHAIN · SOCIAL HEALTH INSURANCE · CONTRIBUTION-LINKED POOLING, AS IN THE EMPLOYEES SYSTEM · GENERAL REVENUE, AS IN THE HEALTH MISSION AND THE · REVENUE COLLECTION TO RISK POOLING TO STRATEGIC PURCHASING TO · INSURANCE LEAVES PREVENTION, MEDICINES AND OUTPATIENT SPENDING UNRESOLVED · CALLING THE TAX-FUNDED PILLAR A CONTRIBUTORY INSURANCE SCHEME

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system	CHAIN → revenue collection to risk pooling to strategic purchasing to provision	TRAP → calling the tax-funded pillar a contributory insurance scheme
TAX-FUNDED → general revenue, as in the health mission and the hospitalisation pillar	GAP → insurance leaves prevention, medicines and outpatient spending unresolved	SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system

CORE CLAIM

- SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system
- TAX-FUNDED → general revenue, as in the health mission and the hospitalisation pillar

MECHANISM

- CHAIN → revenue collection to risk pooling to strategic purchasing to provision
- GAP → insurance leaves prevention, medicines and outpatient spending unresolved

CONSEQUENCE / LIMIT

- TRAP → calling the tax-funded pillar a contributory insurance scheme
- SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: TRAP → calling the tax-funded pillar a contributory insurance scheme.

10

STAGE 10 COMMUNITY HEALTH MAPPING

COMMUNITY HEALTH MAPPING · REACH THE UNDERSERVED FIRST RATHER THAN THE EASIEST TO · VILLAGE HEALTH, SANITATION AND NUTRITION COMMITTEES · INTER-SECTORAL ACTION · WATER, SANITATION, NUTRITION AND GENDER EQUALITY · APPROPRIATE TECHNOLOGY · FRONTLINE HEALTH ACTIVISTS AND AUXILIARY NURSE MIDWIVES · THE UPGRADED WELLNESS CENTRE AS THE NAMED INDIAN DELIVERY

CORE CLAIM	MECHANISM	CONSEQUENCE / LIMIT
EQUITY → reach the underserved first rather than the easiest to reach	INTER-SECTORAL ACTION → water, sanitation, nutrition and gender equality	INSTITUTION → the upgraded wellness centre as the named Indian delivery point
PARTICIPATION → village health, sanitation and nutrition committees	APPROPRIATE TECHNOLOGY → frontline health activists and auxiliary nurse midwives	EQUITY → reach the underserved first rather than the easiest to reach

CORE CLAIM

- EQUITY → reach the underserved first rather than the easiest to reach
- PARTICIPATION → village health, sanitation and nutrition committees

MECHANISM

- INTER-SECTORAL ACTION → water, sanitation, nutrition and gender equality
- APPROPRIATE TECHNOLOGY → frontline health activists and auxiliary nurse midwives

CONSEQUENCE / LIMIT

- INSTITUTION → the upgraded wellness centre as the named Indian delivery point
- EQUITY → reach the underserved first rather than the easiest to reach

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: INSTITUTION → the upgraded wellness centre as the named Indian delivery point.

11

STAGE 11 CAPACITY LIMITS AND ANSWER SPINE

CAPACITY LIMITS AND ANSWER SPINE · UNION FINANCING AND STANDARDS DEPEND ON STATE DELIVERY CAPACITY · WORKFORCE VACANCIES AND UNEVEN CAPACITY LIMIT IDENTICAL NATIONAL SCHEMES · MENTAL HEALTH · A STATUTORY RIGHT AND A TELE-SERVICE STILL NEED DISTRICT · NAME THE COVERAGE DIMENSION TESTED; BUILD · PROVISION PLUS REGULATION · GRADED VERDICT WITH A DASHBOARD-COUNT CAVEAT AND A REVERSAL

CONDITION → Union financing and standards depend on State delivery capacity → GAP → workforce vacancies and uneven capacity limit identical national schemes → MENTAL HEALTH → a statutory right and a tele-service still need district professionals → OPEN → name the coverage dimension tested; BUILD → provision plus regulation → CLOSE → graded verdict with a dashboard-count caveat and a reversal condition

CORE CLAIM

- CONDITION → Union financing and standards depend on State delivery capacity
- GAP → workforce vacancies and uneven capacity limit identical national schemes

MECHANISM

- MENTAL HEALTH → a statutory right and a tele-service still need district professionals
- OPEN → name the coverage dimension tested; BUILD → provision plus regulation

CONSEQUENCE / LIMIT

- CLOSE → graded verdict with a dashboard-count caveat and a reversal condition
- EVIDENCE LIMIT: AUTHORITY / IMPLEMENTATION. Map Union-state-local responsibilities,...

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: CLOSE → graded verdict with a dashboard-count caveat and a reversal condition.

E

EXTRA SUBORDINATE ENRICHMENT — ONLY AFTER THE COMPLETE CORE

OPTIONAL ADVANCED ENRICHMENT · EXPAND HWC/AAM FOR COMPREHENSIVE PRIMARY CARE — REDUCES NEED · STRENGTHEN NHM (FREE DRUGS, DIAGNOSTICS, WORKFORCE) — IMPROVES PUBLIC · PM-JAY FOR SECONDARY/TERTIARY COVER — SHIFTS HOSPITALISATION COST FROM · REGULATORY ENFORCEMENT · CLINICAL ESTABLISHMENTS ACT, PRICE CAPS (NPPA FOR · PUBLIC-PRIVATE PARTNERSHIPS WITH ACCOUNTABILITY · EMPANEL PRIVATE PROVIDERS

OPTIONAL SOURCE DEPTH

- Expand HWC/AAM for comprehensive primary care — reduces need for
- Strengthen NHM (free drugs, diagnostics, workforce) — improves public
- PM-JAY for secondary/tertiary cover — shifts hospitalisation cost from
- Regulatory enforcement: Clinical Establishments Act, price caps (NPPA for

ADVANCED DEBATE

- Public-private partnerships with accountability: empanel private providers
- NHP 2017: Policy, not statute; sets vision for UHC, primary-care-led
- NHM (NRHM + NUHM): Centrally-sponsored scheme; provides flexi-pool funds,
- Ayushman Bharat - HWC/AAM: Target: 1.5 lakh HWCs (upgraded SCs/PHCs);

LEGACY / NUANCE

- Ayushman Bharat - PM-JAY: Its core route uses notified eligibility criteria
- CAUTION: Clinical Establishments Act adoption: Some states have adopted or enacted
- 2024 Q17: The question directly tests the state's role against
- PM-JAY 70+ expansion: The all-citizen 70+ route is operational, regardless of

OPTIONAL STATUS — This optional depth is unnecessary for a competent core answer; use it only for added nuance after the complete core has been written.

END OF MASTER CANVAS • poster embeds this exact master • tiled pages are overlapping crops of the same pixels